

**Laboratory Investigation Report**

Patient Name	: Preeti Bisht	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 33 Y 1 M 22 D /F	OP/IP No	: OP/99799
Max ID/Mobile	: 414926/8752005200	Collection Date/Time	: 31/Aug/2021 12:05PM
Lab ID	: 1266082100722	Receiving Date	: 31/Aug/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 31/Aug/2021

Hematology

SIN No:B2B907060, Test Performed at :896 - Motherland Hospital, Motherland hospital Sec-119

Boo Max Lab - A Division of Max Healthcare Institute Ltd. Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002, 8752 Max Super Specialty Hospital, Saket (West Block)

1, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.
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CIN No.: L72200MH2001PLC322854

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Hematology
CBC (Complete Blood Count)*, Whole Blood EDTA

Date	31/Aug/2021 12:05PM	06/Aug/21 03:07PM	Unit	Bio Ref Interval
Haemoglobin	11.32	11.85	g/dl	12.0 - 15.0
Modified cyanmethemoglobin				
Packed Cell, Volume	38.9	35.1	%	40-50
Calculated				
Total Leucocyte Count (TLC)	4.35	7.30	10~9/L	4.0-10.0
Electrical Impedance				
RBC Count	4.11	3.87	10~12/L	3.8-4.8
Electrical Impedance				
MCV	94.6	90.7	fL	83-101
Electrical Impedance				
MCH	27.5	30.6	pg	27-32
Calculated				
MCHC	29.1	33.8	g/dl	31.5-34.5
Calculated				
Platelet Count	281.7	222.8	10~9/L	150-410
Electrical Impedance				
MPV	11.46	10.99	fL	7.8-11.2
Calculated				
RDW	13.8	14.0	%	11.5-14.5
Calculated				

Differential Cell Count

VCS / Light Microscopy

Neutrophils	69	75	%	40-80
Lymphocytes	24	20	%	20-40
Monocytes	05	03	%	2-10
Eosinophils	02	02	%	1-6
Basophils	00	00	%	0-2

Absolute Leukocyte Count

Calculated from TLC & DLC

Absolute Neutrophil Count	3	5.48	10~9/L	2.0-7.0
Absolute Lymphocyte Count	1.0	1.5	10~9/L	1.0-3.0
Absolute Monocyte Count	0.22	0.22	10~9/L	0.2-1.0
Absolute Eosinophil Count	0.09	0.15	10~9/L	0.02-0.5



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Hematology

Kindly correlate with clinical findings

*** End Of Report ***


Dr. Mrinalini Priyadarshini
DCP, DNB (Path.)
Consultant Pathologist



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Immunoassay

Total - Thyroid Profile*, Serum

Date	31/Aug/2021 12:05PM	Unit	Bio Ref Interval
T3 (Total) CLIA	1.2	ng/mL	0.87-1.78
T4 (Total) CLIA	8.8	µg/dL	6.09-12.23
TSH Chemiluminescence	1.0	uIU/ml	0.34-5.6

Comment

Parameter	Unit	Cord Blood	Adult	1st Trimester	2nd Trimester	3rd Trimester
TSH	uIU/ml	2.3 - 13.2	0.38 - 5.33	0.1 - 2.5	0.2 - 3.0	0.3 - 3.0

Increased in primary Hypothyroidism.
Decreased in primary Hyperthyroidism

Total Thyroid Profile : (Thyroid Function Test, TFT)
T3 (Total), Triiodothyronine
Increase in Hyperthyroidism, and T3 toxicosis,
Decreased in hypothyroidism, states with decreased TBG, and acute and subacute non thyroidal illness
T4(Total) Thyroxine
Increased in Hyperthyroidism, states with increased TBG, Thyrotoxicosis
Decreased in Hyperthyroidism, states with decreased TBG and Strenuous exercise
TSH, Serum : Thyrotropin(Thyroid Stimulating Hormone)
Increased in primary Hypothyroidism.
Decreased in primary Hyperthyroidism.

Note : TSH levels are subject to circadian variation, reaching peak levels between 2 – 4 am and at a minimum between 6 – 10 pm. The variation is of the order of 50% - 206 %, hence time of the day has influence on the measured serum TSH concentrations.
TSH assay is strandized to the 3rd generation for human TSH.
The Cyclical variations may be quite large; therefore the timing of specimen collection must be strictly controlled.
Advise : Kindly do Thyroid Profile/TSH in morning hours only.



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Immunoassay**Prolactin, Serum***

Date	31/Aug/2021 12:05PM	Unit	Bio Ref Interval
Prolactin CLIA	19.62	ng/mL	

Ref Range**Males :** 2.64 - 13.13**Females :**

Premenopausal

(<50 years of age): 3.34 - 26.74

Postmenopausal

(>50 years of age): 2.74 - 19.64

Interpretation

Increased in prolactin-secreting pituitary tumors, amenorrhea and/or galactorrhea, Chiari-Frommel and Argonz Del Cstillo syndromes, various types of hypothalamic-pituitary disease (e.g. sarcoidosis, granulomatous diseases, craniopharyngioma, metastatic cancer, empty sella syndrome), primary hypothyroidism, anorexia nervosa, polycystic ovary syndrome, renal failure, insulin-induced hypoglycemia, chest wall injury, adrenal insufficiency, and pituitary stalk section surgery

Decreased in pituitary apoplexy (Sheehan's Syndrome)

Kindly correlate with clinical findings

***** End Of Report *****

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